# Ideation Phase
# Define the Problem Statements

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| Date | 31 January 2026 |
| Team ID | 0827CY231062 |
| Project Name | Health360: Executive Analytics & Population Health Dashboard |
| Maximum Marks | 2 Marks |

## Customer Problem Statement Table

| Problem Statement (PS) | I am (Customer) | I'm trying to | But | Because | Which makes me feel |
|---|---|---|---|---|---|
| PS-1 | A Chief Medical Officer (Dr. Michael Carter) managing clinical risk across a 5,000-patient cohort. | Identify high-risk cardiovascular patients and evaluate chronic disease trends (Diabetes, Heart Disease, Stroke). | Static reports lack correlation between vitals (BP vs. Cholesterol) and patient outcomes. | Patient data is stored in isolated tabular spreadsheets without visual risk stratification tools. | **Frustrated & concerned** about delayed clinical intervention and missing early risk warnings. |
| PS-2 | A Director of Wellness Programs (Emily Rodriguez) designing preventive lifestyle intervention. | Target high-risk behavioural cohorts by analysing interactions between exercise, BMI, smoking, and alcohol use. | It is difficult to isolate which specific combination of lifestyle habits drives the highest composite health risk. | Traditional reporting provides fragmented summaries instead of multi-variable cross-tabulated risk matrices. | **Overwhelmed & Inefficient** when trying to allocate wellness resources where they are needed most. |
| PS-3 | A Head of Population Health (Dr. Priya Nair) overseeing demographic distributions and health resource allocation. | Monitor population-wide weight stratifications, age brackets (Children to Elder), and primary symptom prevalence (Chest Pain, Dizziness, Fatigue). | Static demographic breakdowns don't allow instant cross-filtering by age group or symptom severity. | Population data is split across disjointed operational summaries without unified dynamic visual filters. | **Handicapped & Unprepared** when attempting to forecast facility staffing and resource demand accurately. |